

D222

Comprehensive Health Assessment

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A nurse at a clinic with extended evening hours is assessing the abdomen of a 45-year-old accountant who reports mild cramping. The patient ate lunch an hour ago. After inspection, what should the nurse do next?

- A. Skip auscultation because the patient ate recently.
- B. Palpate all four quadrants.
- C. Auscultate bowel sounds.
- D. Percuss the abdomen.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Why is the abdomen auscultated before it is palpated?

- A. To measure liver size before listening.
- B. Percussion and palpation can change the frequency of bowel sounds.
- C. Auscultation always comes first in every body system.
- D. Palpation is painful, so it is always done at the very end of the exam.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A nurse on a unit that recently remodeled its rooms admits a new patient. The patient is a retired firefighter. Which finding is subjective data?

- A. 2+ pitting edema in both ankles
- B. The patient states, "My chest feels tight when I climb stairs."
- C. Fidgeting and pacing observed by the nurse
- D. Blood pressure 148/92

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Which of these is objective data?

- A. Reports nausea since this morning
- B. Rates pain 7 out of 10
- C. Respiratory rate 24 with use of neck muscles to breathe
- D. Says she feels dizzy when standing

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A nurse at a busy urgent care with a new triage system has four patients waiting. Which patient should be seen first?

- A. A patient with a likely broken wrist who rates pain 9 out of 10
- B. A patient with diabetes whose glucose is 250 with no symptoms
- C. The patient who has waited the longest, two hours
- D. A patient with a swollen lip and tongue and a hoarse voice after eating shrimp

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A patient in the trauma bay at a regional hospital that recently expanded its helipad opens eyes only to pain (2), makes incomprehensible sounds (2), and withdraws from pain (4). The patient's helmet was cracked. What is the Glasgow Coma Scale score?

- A. 8
- B. 16
- C. 6
- D. 4

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

A patient's total GCS is 11. Eye opening to voice scores 3, and confused speech scores 4. What is the motor score, and what does it describe?

- A. 4 — withdraws from pain
- B. 6 — obeys commands
- C. 7
- D. 4 — obeys commands

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A 70-year-old patient at a hospital that recently switched meal vendors is resting quietly and watching TV, but rates her post-op pain 8 out of 10. Her vital signs are normal. Her daughter says she “always complains.” What is the best interpretation?

- A. Accept her rating as valid, treat per orders, and reassess.
- B. Older adults always feel less pain.
- C. Her pain cannot be 8 with normal vital signs.
- D. Rely on the daughter’s view that she overstates pain.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A nurse at a clinic that serves many immigrant families and has a new wheelchair ramp assesses a patient who wears an amulet around his neck. The nurse once read that people from the patient’s home region distrust Western medicine. What is the best approach?

- A. Remove the amulet before the exam without asking.
- B. Ask the patient what the amulet means to him and about his health beliefs and preferences.
- C. Assume he distrusts Western medicine and keep explanations short.
- D. Skip cultural questions to save time.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

During a routine visit at a clinic with a busy pediatric wing, a 52-year-old patient says, “Lately I feel like everyone would be better off without me.” He recently finished a marathon. What should the nurse do next?

- A. Schedule a follow-up visit in three months.
- B. Reassure him that finishing a marathon shows he is healthy.
- C. Ask him directly whether he is thinking about suicide, and follow the clinic’s safety protocol.
- D. Avoid asking directly so the idea is not suggested.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A nurse on a pediatric unit with a new playroom hears a harsh, high-pitched sound on inspiration from a toddler's upper airway, audible without a stethoscope. The toddler is holding a stuffed bear. What is the sound, and how urgent is it?

- A. Fine crackles from fluid in the alveoli
- B. Wheezing from the lower airways
- C. Rhonchi that will clear with coughing; low priority
- D. Stridor; it signals upper airway narrowing and needs urgent attention.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12. [TRANSFER — reversed stem / different variable]

Which description matches wheezes?

- A. A low-pitched grating sound from inflamed pleura
- B. A harsh inspiratory sound from the upper airway
- C. Fine popping sounds from fluid-filled alveoli
- D. High-pitched, musical sounds from narrowed lower airways, often on expiration

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A nurse at a clinic ~~with extended evening hours~~ is assessing the abdomen of a 45-year-old accountant who reports mild cramping. ~~The patient ate lunch an hour ago.~~ After inspection, what should the nurse do next?

Target word: next step in abdominal assessment **Decides:** Auscultation comes before touching the abdomen.

A	Camouflage	A recent meal does not remove a step.
B	Inverted operation	Palpation comes last for the abdomen.
C	Correct	Listening before percussion and palpation avoids altering bowel sounds.
D	Dropped element	Skips auscultation.

2. Answer B Concept 1 • transfer

Why is the abdomen auscultated before it is palpated?

Target word: reason for the order **Decides:** Touch can change bowel sounds.

A	Inverted operation	Liver span is found by percussion, after auscultation.
B	Correct	Listening first gives an accurate baseline.
C	Absolute language	Most systems use inspect, palpate, percuss, auscultate.
D	Adjacent category	Tender areas are palpated last, but that is not why auscultation comes first.

3. Answer B Concept 2

A nurse on a unit ~~that recently remodeled its rooms~~ admits a new patient. ~~The patient is a retired firefighter.~~ Which finding is subjective data?

Target word: subjective data **Decides:** Only the patient can report it.

A	Confusion set	Observed and graded by the nurse = objective.
B	Correct	A symptom described by the patient.
C	Adjacent category	Observed behavior is objective, even when it suggests a feeling.
D	Confusion set	Measured by the nurse = objective.

4. Answer C Concept 2 • transfer

Which of these is objective data?

Target word: objective data **Decides:** Measured or observed by the nurse.

A	Inverted operation	A reported symptom is subjective.
B	Confusion set	A number, but still self-reported.
C	Correct	Counted and observed by the nurse.
D	Confusion set	Patient-reported = subjective.

5. Answer D Concept 3

A nurse at a busy urgent care ~~with a new triage system~~ has four patients waiting. Which patient should be seen first?

Target word: priority patient **Decides:** Possible airway compromise.

A	Adjacent category	Severe pain matters but ranks after airway.
B	Confusion set	Abnormal but not immediately life-threatening.
C	Camouflage	Wait time does not set clinical priority.
D	Correct	Signs of airway swelling come first under ABCs.

6. Answer A Concept 4

A patient in the trauma bay at a regional hospital ~~that recently expanded its helipad~~ opens eyes only to pain (2), makes incomprehensible sounds (2), and withdraws from pain (4). ~~The patient's helmet was cracked.~~ What is the Glasgow Coma Scale score?

Target word: total GCS **Decides:** Add the three subscores.

A	Correct	$2 + 2 + 4 = 8$, indicating severe impairment.
B	Inverted operation	Multiplies instead of adding ($2 \times 2 \times 4$).
C	Dropped element	Leaves out the verbal score.
D	Wrong stem number	That is only the motor score.

7. Answer A Concept 4 • transfer

A patient's total GCS is 11. Eye opening to voice scores 3, and confused speech scores 4. What is the motor score, and what does it describe?

Target word: solve for the motor subscore **Decides:** $11 - 3 - 4 = 4$.

A	Correct	Motor 4 on the 1-6 scale is withdrawal.
B	Adjacent category	Would make the total 13.
C	Inverted operation	Adds $3 + 4$ instead of subtracting from 11.
D	Confusion set	Right number, wrong meaning; obeying commands is 6.

8. Answer A Concept 5

A 70-year-old patient at a hospital ~~that recently switched meal vendors~~ is resting quietly and watching TV, but rates her post-op pain 8 out of 10. Her vital signs are normal. ~~Her daughter says she "always complains."~~ What is the best interpretation?

Target word: how to read the pain report **Decides:** Self-report is the most reliable indicator.

A	Correct	Pain is what the patient says it is.
B	Absolute language	Aging does not reduce pain; it is often undertreated.
C	Adjacent category	Vital signs often normalize with ongoing pain.
D	Camouflage	A family opinion does not outweigh self-report.

9. Answer B Concept 6

A nurse at a clinic that serves many immigrant families ~~and has a new wheelchair ramp~~ assesses a patient who wears an amulet around his neck. The nurse once read that people from the patient's home region distrust Western medicine. What is the best approach?

Target word: culturally sound assessment **Decides:** Ask the individual rather than assume.

A	Inverted operation	Disregards the patient's beliefs and consent.
B	Correct	Individual assessment avoids stereotyping.
C	Absolute language	A group generalization is not an assessment of this patient.
D	Dropped element	Leaves out part of a comprehensive assessment.

10. Answer C Concept 7

During a routine visit at a clinic ~~with a busy pediatric wing~~, a 52-year-old patient says, "Lately I feel like everyone would be better off without me." ~~He recently finished a marathon.~~ What should the nurse do next?

Target word: next assessment step **Decides:** A possible suicide warning sign.

A	Dropped element	Skips assessment of immediate risk.
B	Camouflage	Physical fitness does not address the statement.
C	Correct	Direct asking is standard; it does not plant the idea.
D	Inverted operation	A common myth; asking is protective.

11. Answer D Concept 8

A nurse on a pediatric unit ~~with a new playroom~~ hears a harsh, high-pitched sound on inspiration from a toddler's upper airway, audible without a stethoscope. ~~The toddler is holding a stuffed bear.~~ What is the sound, and how urgent is it?

Target word: name the sound and priority **Decides:** Inspiratory, upper airway, harsh = stridor.

A	Confusion set	Crackles are popping sounds heard with a stethoscope.
B	Confusion set	Wheezes are musical and usually expiratory.
C	Adjacent category	Rhonchi are low-pitched and from secretions, not an upper airway sound.
D	Correct	Stridor is an airway emergency sign.

12. Answer D Concept 8 • transfer

Which description matches wheezes?

Target word: wheeze features **Decides:** Musical sound from narrowed lower airways.

A	Adjacent category	That is a pleural friction rub.
B	Confusion set	That is stridor.
C	Confusion set	That is crackles.
D	Correct	Classic wheeze description.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	10	
Camouflage	Answers a stem fact that was never the question.	4	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Abdominal assessment order	For the abdomen: inspect, auscultate, percuss, then palpate, because touching can change bowel sounds.	Using the general inspect–palpate–percuss–auscultate order.	Abdomen and “what next?”
2	Subjective vs. objective data	Subjective = what the patient says (symptoms, self-rated pain); objective = what the nurse observes or measures.	Treating a patient’s numeric pain rating as objective.	“States,” “reports” vs. measured or observed.
3	Prioritizing cues with ABCs	Airway threats come before breathing, circulation, and everything else, including severe pain.	Seeing the patient with the worst pain or longest wait first.	Several patients and “who first?”
4	Glasgow Coma Scale	GCS = eye (1–4) + verbal (1–5) + motor (1–6); range 3–15; 8 or less signals severe injury.	Adding wrong subscales or misreading the motor scale.	Three responses to score.
5	Pain self-report	The patient’s self-report is the most reliable pain indicator; normal vital signs or calm behavior do not rule out pain.	Discounting pain because the patient looks comfortable.	A high rating with calm behavior or normal vitals.
6	Cultural assessment	Ask the patient about beliefs and practices; do not assume based on group generalizations.	Stereotyping from something read about a group.	A cultural item or practice and a generalization.
7	Suicide risk screening	When a patient expresses hopelessness, ask directly about suicidal thoughts; asking does not plant the idea.	Avoiding the question or reassuring without assessing.	“Better off without me” or similar statements.
8	Adventitious breath sounds	Stridor = harsh inspiratory upper-airway sound (urgent); wheezes = musical, narrowed lower airways; crackles = popping from fluid in alveoli.	Mixing up stridor and wheezes.	Pitch, timing, and location of a sound.

Course scope (public description): Comprehensive Health Assessment builds upon students’ existing knowledge of nursing assessment. The course presents current and innovative assessment techniques of the physical, mental, emotional, and spiritual well-being of patients. Use of assessment data and shared decision-making are discussed throughout the course. This course also outlines the concepts of a head-to-toe assessment, providing students with an understanding of how to critically think about the different aspects of the assessment and analyze patient cues to determine the implications of findings. Students will also analyze lifestyle and cultural implications of health. All prior courses in the sequence for this program serve as prerequisites for this course.